

A&E four-hour performance

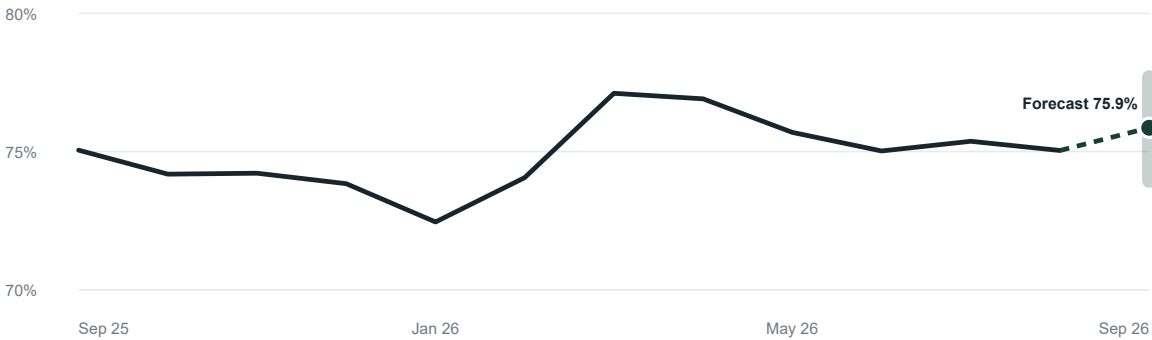
Forecast for September 2026 · data through August 2026 · issued 2026-09-25

National all-types performance fell by 0.7pp over the three months to August 2026, reaching 75.0%. The September 2026 forecast is 75.9%, 0.8pp above the latest result. The model's expected improvement is mainly driven by the underlying trend; the seasonal effect is small. The reference projection reaches 77.0% by March 2027, 5.0pp below the 82% interim milestone.

LATEST · AUGUST 2026 75.0% Baseline when outlook was made	OUTLOOK · SEPTEMBER 2026 75.9% 80% range 74.0%–77.7%	ACTUAL The published result will be added here without removing the forecast. 9.1pp below the 85% 2028/29 milestone
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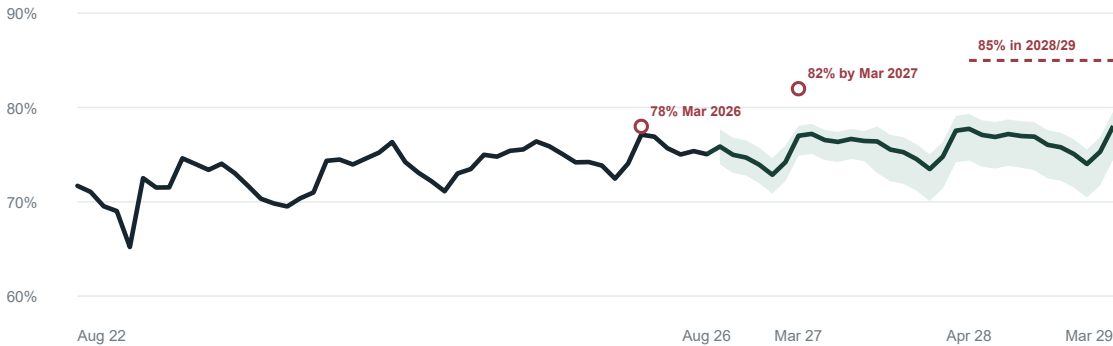
Recent performance and next release

The vertical band is the empirical 80% expected range.



Longer-term outlook and milestones

Descriptive projection; not forced to meet targets.



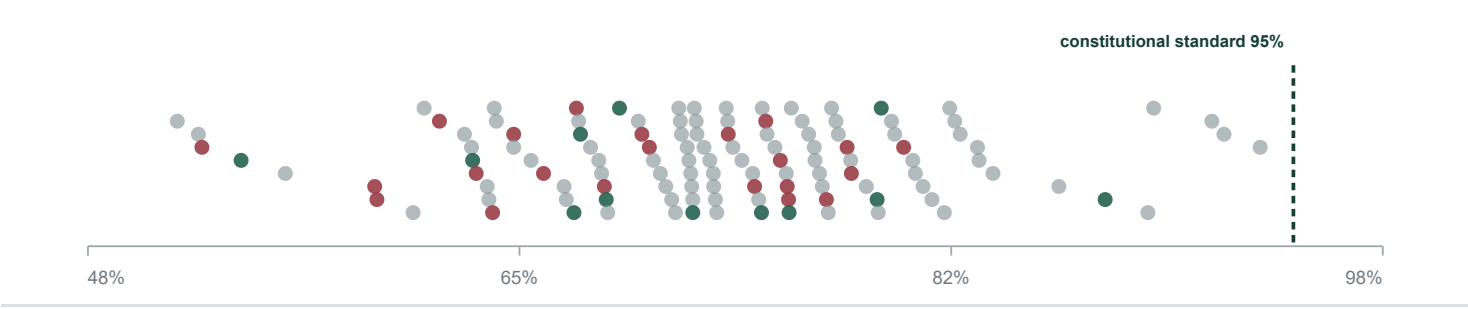
Benchmark: 95% constitutional standard; planning milestones shown on the chart.
Data: NHS England A&E attendances and emergency admissions. **Targets:** Medium Term Planning Framework.
Method: Fixed equal-weight ensemble of same-month persistence, damped annual drift and a shared-season current-era trend; all three components are required. The final fit uses 190 months. Accuracy is tested on 15 rolling one-month-ahead forecasts. The 80% range uses 78 empirical current-era forecast errors. The pre-release commentary compares the seasonal component with the two trend-bearing components. Provider distribution and trajectory watch are on page 2.

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Distribution and trajectory watch · August 2026

Provider distribution

Latest reported performance across providers. This is a comparison of current levels, not a measure of whether each provider is improving or deteriorating.



Trajectory watch

Signals compare six successive results with forecasts made before each release. A low performer can be improving, and a high performer can be deteriorating. The archive is still building: each signal states whether it uses archived release forecasts, a historical backtest, or both.

120 providers assessed	12 sustained favourable	22 sustained adverse	86 without an active signal
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Sustained adverse signals outnumber favourable signals (22 versus 12). Most providers (86 of 120) do not meet the active six-release threshold.

Largest sustained favourable moves	Largest sustained adverse moves
George Eliot Hospital NHS Trust Latest 87.7% · 6 of 6 months in this direction · Continuing historical backtest Historically unusual +13.9pp SIX-MONTH GAP	Airedale NHS Foundation Trust Latest 59.4% · 5 of 6 months in this direction · Continuing historical backtest Historically unusual -8.0pp SIX-MONTH GAP
North Cheshire And Mersey NHS Foundation Trust Latest 75.5% · 6 of 6 months in this direction · Continuing historical backtest Historically unusual +9.0pp SIX-MONTH GAP	Torbay And South Devon NHS Foundation Trust Latest 63.4% · 6 of 6 months in this direction · Continuing historical backtest Historically unusual -7.1pp SIX-MONTH GAP
Nottingham University Hospitals NHS Trust Latest 67.4% · 6 of 6 months in this direction · Continuing historical backtest Historically unusual +8.0pp SIX-MONTH GAP	Dorset County Hospital NHS Foundation Trust Latest 76.9% · 6 of 6 months in this direction · Continuing historical backtest Historically unusual -6.0pp SIX-MONTH GAP

Trajectory-watch rule. The average 6-release forecast error must be at least 2.0pp, at least 5 of 6 releases must point in the same direction, and the latest release must confirm it. These are screening prompts—not league tables, causal estimates or judgements of provider quality.